

An Introduction to *EPISODES OF CARE*

What is an Episode of Care (EOC)?

TennCare has defined an episode of care as “health care delivered during a specified time period to treat a physical or behavioral condition” For both included behavioral health diagnoses an episode has been defined as 6 months (which can be repeated). For each episode there are defined quality measures and acceptable costs.

What diagnoses are covered by EOC?

Currently the *only* mental health diagnoses covered by Episodes of Care are the diagnoses associated with **ADHD** and **ODD**.

What does this mean for Centerstone?

If our average cost per episode is in the “Not Acceptable” range, we pay a penalty of 50% of the difference in our cost and the not acceptable threshold times the total number of cases (regardless of quality metric scores). Pharmacy costs are included in total costs.

What does this mean for me?

- Make sure your primary diagnosis is correct. We don’t want to include costs for treating other disorders.
- Include all appropriate comorbid diagnoses.
 - Some diagnoses, including some social drivers of health, can help us by adjusting costs down.
 - Some diagnoses, deemed to significantly impact costs, can exclude an episode from the cost limits.
- Provide the appropriate level of service to address the diagnosed conditions including individual and family therapy and referrals for medication management and HealthLink as appropriate.
- Review cases that have been identified as cost outliers when asked to do so.
- When seeing a client with ADHD or ODD make sure to designate the diagnosis that is the principal reason for the visit. This allows the costs of the session to be attributed to the correct diagnosis.

NOTE: when doing a session note, you cannot choose as the primary dx either a Z-code or a diagnosis that includes “due to another medical condition” as these diagnoses are not billable at this time.

Are you saying I should limit how much I see the client to save money?

We want you to use good clinical judgment regarding what care is needed and we want every client to be able to get all the care they need. Attributing the costs to the right area and capturing appropriate risk adjustments and exclusions allows us to be able to provide needed care while minimizing our financial risk.

What diagnoses should I consider?

Only the ones that apply to your client! Carefully look at onset, duration, intensity, and domains of behaviors and symptoms. Consult with your clinical supervisor if necessary and add the ones that apply to your client. Some of them may be ones included in the list of exclusionary diagnoses, but that is not the important part. The important part is accurately diagnosing your client so that you can create a care plan that meets their individual needs.

What is considered to be quality care?

For both ADHD and ODD diagnoses, the research shows that consistent and comprehensive **family therapy along with individual therapy** (and medication management as indicated) is best practice. It is often difficult to involve caregivers (especially in school-based services) but it is important. ADHD therapy should include parent education related to how their child thinks and operates differently from what the parent may expect, and to teach them how to help their child develop coping skills and routines that will result in better outcomes. ODD is not just behavioral, but also relational, and family therapy allows for the caregiver to be involved in the changes their child is making, as well as be a conduit for change. Working with the caregiver to provide positive reinforcement at home promotes lasting changes in child behavior.